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US Healthcare Provider Credentialing Research Report

A credentialing operations team needs separate evidence for provider identity, authority to practice, credentialing review and enrollment. This report maps each decision to its source and shows how an analyst can move an application forward while keeping unresolved evidence visible.

Scope and research method

The research centers on individual physicians, using California for licensure, Medicare fee-for-service Part B for enrollment, and scoped examples from Medicare Advantage, Medi-Cal and UnitedHealthcare. Public sources were reviewed on November 10, 2026. The source register identifies the issuing organization, finding and limit of each reference. Analyst controls in this report are portfolio procedures.

Operational evidence and decisions

Step	Analyst action	Record for handoff
Intake	Identify the provider, legal entity, state, site, payer or product, and application action.	Case ID, owner, evidence checklist and due dates.
Verification	Compare submitted facts with the issuer or appropriate registry. Keep each verification result separate.	Source, lookup date, subject match, observed result and evidence reference.
Review	Route unresolved identity, status, restriction or missing-evidence issues to the designated reviewer.	Disposition, decision owner and next action.
Submission	Use the program route and authorized signer. Track receipt and requests for additional information.	Packet version, receipt or tracking ID, notice and response due date.
Close and monitor	Reconcile the decision, effective date and scope. Set the next monitoring action for the case.	Decision evidence, release reviewer and monitoring owner.

Terms used in this report

Licensure is state authorization for the relevant profession. Credentialing evaluates qualifications against an organization’s criteria. Privileging addresses what clinical services the practitioner may perform for a facility. Enrollment concerns participation or billing for a specific program. A network contract is a separate participation step when the payer requires it. [S01, S05, S06, S08, S13]

Licensure Identity and Screening

State license verification

Start with the issuing board for the profession and state. For a California MD, use the Medical Board of California's link to the public search and resolve the result against the provider's name, license type and license number. Record the displayed status, expiration if shown, relevant public actions, date checked and evidence reference. California directs osteopathic physician inquiries to a separate board. [S01, S02]

Read any linked public order before summarizing a restriction. If the result is missing, ambiguous or conflicts with the intake record, preserve the discrepancy and seek board clarification or certified verification when required by the receiving organization. Limit any summary of absent actions to the scope of the public profile and the date reviewed. [S01, S02]

NPI identity verification

Compare the submitted 10-digit NPI with the NPPES Registry. Check entity type, name, and relevant taxonomy or practice details. Keep the individual Type 1 and organization Type 2 records separate. A difference in address or taxonomy warrants context and follow-up because those fields describe a registry record. NPI issuance does not establish licensure or credentialing. [S03, S04]

Screening and decision ownership

Source	Use	Boundary
OIG exclusion list [S07]	Check plausible name variants and route possible matches for authorized identity confirmation.	Keep restricted identifiers out of the tracker. Escalate ambiguous results.
National Practitioner Data Bank [S06]	Route a query through the eligible, registered organization for its permitted purpose.	Public access is limited to guidance. Reports require authorization.
Medicare Advantage [S05]	Under the cited rule, covered initial credentialing includes primary-source verification of licensure or certification. Recredential at least every three years within scope.	Apply the documented process for contracted providers and suppliers covered by the rule.

Credentialing file assembly

Build the checklist from the organization's documented criteria. Route education, residency or specialty certification to the issuing institution or board when called for. The analyst compiles evidence and flags exceptions. The designated decision maker evaluates the file. Keep a pending decision when the subject cannot be uniquely identified or a public order needs interpretation. [S05, S13]

Enrollment Routes and Decision Evidence

Select the payer, program, provider type, location and action before collecting application fields. Initial enrollment, updates, revalidation and reassignment can follow different instructions. Preserve the source or checklist version used for the packet. [S08, S12, S13]

Route	Public workflow	Control for the case file
Original Medicare Part B [S08]	CMS identifies PECOS as the online route and lists CMS-855I for physicians and non-physician practitioners and CMS-855B for applicable groups and suppliers.	Confirm individual versus group and reassignment needs, signer authority, attachments and receipt.
California Medi-Cal [S11, S12]	The Medicaid rule addresses state screening. DHCS directs applicable individual and group MD applicants to PAVE.	Use the DHCS checklist for the applicant category. Confirm any rendering or group linkage required for the case.
UnitedHealthcare medical network [S13]	The published onboarding workflow includes credentialing and contracting. The payer describes primary-source verification for completed applications.	Confirm the product, contractual participation and effective date or go-live instructions before release.

Manage development requests

Record the request date, requested items, notice reference, response owner and external deadline. Separately schedule an internal follow-up. For Medicare enrollment, 42 CFR 424.525(a)(1) allows rejection when complete missing information is not provided within 30 calendar days of the contractor’s request. Other grounds and clocks appear in the same section. Document any extension confirmed by the contractor. [S09]

Reconcile the approval before release

Keep the submission receipt, decision notice and billing effective date in separate fields. For physicians under 42 CFR 424.520(d), the effective date is the later of the filing date of the application that is subsequently approved or the date the provider first furnished services at the new practice location. Use the decision notice to confirm the applicable identity, location and effective date. Send conflicts to the contractor and hold downstream release until clarified. [S08, S10]

Maintain the approved record

Assign an owner to monitor license expirations, reportable changes and upcoming revalidation. Check the CMS Revalidation List and the applicable MAC notice for Medicare. Review payer and board instructions before applying any other maintenance cadence. [S01, S08, S13]

Continuing Medical Education Review

Treat CME as a renewal requirement that depends on the issuing board, license type, renewal period and any applicable exceptions. A course certificate is evidence for a specific learning activity. A renewal-readiness decision requires checking the full applicable requirements. [S14, S15, S16]

Two examples of board-specific handling

Board scope	Research finding	Analyst implication
California Medical Board physician and surgeon [S14]	The Board states a baseline of 50 approved hours per biennial renewal cycle. It describes a four-year minimum record retention period and asks for documentation when selected for audit.	Confirm the renewal window, recognized education provider, applicable topics and any waiver. Retain a policy-compliant evidence reference.
Texas Medical Board CE Broker FAQ [S15]	For renewal on or after September 1, 2026, the Board says licensees must have an active CE Broker account and upload proof of the CE required for renewal.	Confirm license-specific credit and exception rules separately from the reporting method.

Checklist for a course document

Capture the learner, course title, completion date, education provider, claimed hours or credits, credit type and any required topic. Compare the date with the renewal window. If a claimed designation or accreditation is unclear, use the education provider and appropriate accreditor to verify it. The AMA describes who may certify activities for AMA PRA Category 1 Credit™. ACCME lists accredited organizations and updates its directory monthly. [S14, S16, S17]

Record the extent of review

Use separate states for certificate received, certificate reviewed, renewal-readiness evidence complete and board action when relevant. A missing item leaves the client review open. Document what is missing, who owns it and when to follow up. The portfolio’s simulated client uses an internal CME document gate before submission in one case. The Medicare submission guidance is a separate source. [S08, S14]

Source Register for Verification

Accessed November 10, 2026. Each underlined source name links to the official public URL. Source IDs identify the supporting references in this report.

S01 Medical Board of California physician license lookup

Directs users to the public license search and identifies the separate board for osteopathic physicians. For the selected license and jurisdiction. A lookup is a dated observation.

S02 Medical Board of California lookup disclosure

Describes the information that can appear in the public profile and limits on disclosure. Absence of public information supports only a statement about the profile reviewed.

S03 CMS NPPES NPI Registry

Public NPI identity lookup. Use it to review the entity type, name and available record details. Registry data are specific to NPPES. Confirm licensure with the issuing board.

S04 CMS National Provider Identifier fact sheet

Explains the 10-digit NPI and individual and organization entity types. CMS explicitly separates enumeration from licensing and credentialing.

S05 42 CFR 422.204 Provider selection and credentialing

Defines documented Medicare Advantage credentialing and recredentialing requirements within its scope. The requirement to recredential at least every three years applies to the covered MA process.

S06 HRSA NPDB access FAQ

Explains who may view reports, authorized reasons for queries and the self-query option. Public guidance is accessible. Individual reports are restricted.

S07 HHS OIG LEIE search instructions

Explains search variants and identity verification for a potential exclusion match. A name match is a lead. Resolve identity in an authorized workflow.

S08 CMS Medicare Provider Enrollment MLN9658742

Explains PECOS, enrollment actions, application types, signatures, contractor review and revalidation. Use the applicable program and current contractor instructions for the actual filing.

Source Register for Enrollment and CME

Accessed November 10, 2026. For live casework, consult the current version and the notice for the specific provider, program, product and location.

S09 42 CFR 424.525 Medicare enrollment application rejection

Covers rejection grounds, including the 30-calendar-day missing-information provision, and discretionary extensions. The clocks depend on the issue. Keep the actual notice and verify any extension.

S10 42 CFR 424.520 Medicare billing effective date

Sets billing effective-date rules by provider and supplier type, including physicians under paragraph (d). Use the case-specific decision and escalate discrepancies. Retrospective billing is a separate question.

S11 42 CFR 455.410 Medicaid enrollment and screening

Requires state Medicaid agencies to screen enrolled providers under the subpart. State and provider-type guidance supplies the implementation details.

S12 California DHCS Doctor of Medicine application

Directs applicable individual and group MD applications to PAVE and lists required documents as applicable. California Medi-Cal example. Other provider types and enrollment categories have separate instructions.

S13 UnitedHealthcare medical provider network onboarding

Describes Onboard Pro, primary-source verification, credentialing and contracting stages. Illustrates one payer workflow. Confirm the product, state and provider scope.

S14 Medical Board of California CME guidance

Describes the biennial baseline, approved education, record retention, audits and waiver considerations. Confirm physician status, cycle and any applicable topic or waiver before advising a provider.

S15 Texas Medical Board CE Broker renewal FAQ

Describes the CE Broker verification requirement for licensees renewing on or after September 1, 2026. The FAQ addresses the reporting mechanism. Consult current rules for license-specific credits and exceptions.

S16 AMA PRA credit system requirements

Describes which accredited organizations can certify activities for AMA PRA Category 1 Credit™. Credit designation does not establish a licensee's completion or the board's acceptance for a specific requirement.

S17 ACCME CME provider directory

Lists organizations accredited in the ACCME system and describes monthly refreshes. Verify course-level details with the education provider and the applicable board.